ORAL CAVITY MALIGNANCY

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PATIENT'S PARTICULARS

- **Name – Mr.X**
- **Age – 62 years**
- **Occupation – laborer**
- **Address – Gandhi chowk , Islampur**
- **Socio economic status – class 4 (Modified B G Prasad classification)**
- **Education – 10th standard**
- **Date of examination – 04-03-2020**

CHIEF COMPLAINT

An ulcer in the inner aspect of the left cheek since 5 months.

HISTORY OF PRESENT ILLNESS

- Apparently well 5 months back
- An ulcer in the inner aspect of his left cheek which was insidious in onset and rapidly progressive in size. Initially, the ulcer was over the inner left cheek and later it grown beyond it to involve the angle of mouth to increase from a size of 2 x 2 cms to 6 x 3 cms now.
- Initially, it was painful but later on became painless

- **He chews tobacco since past 20 years.**
- **He uses a tobacco quid that he keeps in the inner aspect of the left cheek for about 30 mins each time and spits it out.**
- **He uses such a quid about 6 times a day.**

- **It is not associated with loosening of teeth**
- **No history of (h/o) Bleeding or discharge**
- **No h/o Constitutional symptoms**
- **No h/o difficulty in chewing /swallowing or speech.**
- **No h/o restriction in mouth opening**
- **No h/o loss of appetite or significant unintensional weight loss**

- **No h/o radiation exposure**
- **No h/o trauma**
- **No h/o multiple sexual partners or IV drug abuse**
- **No history suggestive of distant metastasis**

PAST HISTORY

- Non-diabetic and non-hypertensive
- No history of chronic illness such as Tuberculosis
- No history of any medical or surgical interventions or any dental operations
- No history of any allergy
- No history of any drug intake

PERSONAL HISTORY

- Diet – Non-vegetarian
- Bladder habits - normal
- Bowel habits – normal
- Sleep pattern – normal
- Chews tobacco for past 20 years
- Occasional intake of alcohol

FAMILY HISTORY

No any significant history of head and neck malignancies in the first degree relatives

SUMMARY

A 62 year old man presented with chief compliant of a painless ulcer in the inner aspect of left cheek . The ulcer was insidious onset and initially limited to left cheek but later progressing to involve the angle of mouth with increase in size from 2 x 2 cm to 6 x 3 cm. He chews tobacco since past 20 years and drinks alcohol occasionally.

GENERAL EXAMINATION

- Examined in sitting posture with adequate day light with proper informed consent.
- Patient is conscious,co-operative and well oriented with time, place and person
- Karnofsky performance score = 90
- Well nourished with BMI – 20.4 kg/m2
- Well hydrated , Afebrile

Initial questions	Follow-up questions	Symptom characterization	KPS %	Comments
Is the patient able to carry on with his/her normal work or activity?				
YES	A Does the patient have symptoms? (pain, loss or gain of weight, reduced energy etc.)	No symptoms.	100	Normal, no complaints, no evidence of disease.
		Mild symptoms.	90	Able to carry on normal activity, minor signs or symptoms of disease.
		Moderate symptoms.	80	Normal activity with effort, some signs or symptoms of disease.
NO	B Does the patient need assistance? (grooming, food intake, dressing, other daily activities)	No assistance.	70	Cares for self, unable to carry on normal activity or to do active work.
		Occasional assistance.	60	Requires occasional assistance, but is able to care for most of his needs.
NO		Considerable assistance.	50	Requires considerable assistance and frequent medical care.
Is the patient bedridden for more than half a day?				
YES	C What is the patient's degree of disability in terms of bed confinement?	Bedridden in more than 50 % of the time.	40	Disabled, requires special care and assistance.
		Almost completely bedridden.	30	Severely disabled, hospitalization is indicated although death not imminent.
		Completely bedridden and dependent upon extensive nursing care by professionals and/or family.	20	Hospitalization necessary, very sick, active supportive treatment necessary.
		Completely bedridden *and* comatose or barely arousable.	10	Moribund, fatal processes progressing rapidly.
		Dead.	0	Dead.

- **Pulse rate – 86 beats/min on right radial , regular , normal volume and character**
- **Blood pressure – 126/72 mmHg on right side in lying down position**
- **Respiratory rate – 16 breaths/min**
- **No pallor/Icterus/Cyanosis/Edema/clubbing**

REGIONAL EXAMINATION

On Inspection –

- **Mouth opening was adequate.**
- **Facial symmetry was intact.**
- **A solitary ulceroproliferative growth in the left buccal mucosa**
- **Size – 6 x 3 cm**
- **Shape – irregular**
- **Margin – irregular**

- **Extent –**
- **Anteriorly – extending upto the anterior commissure and lies 1 cm from the upper and lower gingivobuccal sulci.**
- **Posteriorly – upto the left lower 3rd molar, lying 5 mm from the retromolar trigone.**
- **Edge – rolled out**
- **Floor – reddish with slough and necrotic material**

- **Poor oral hygiene depicted with tobacco stains and halitosis.**
- **There were few leukoplakic patches over the gums, soft palate and hard palate and submucous fibrosis surrounding the lesions.**
- **The right side buccal mucosa is normal.**
- **There was no loosening of teeth.**
- **Tongue protrusion was normal and the openings of parotid ducts were normal.**





On palpation –

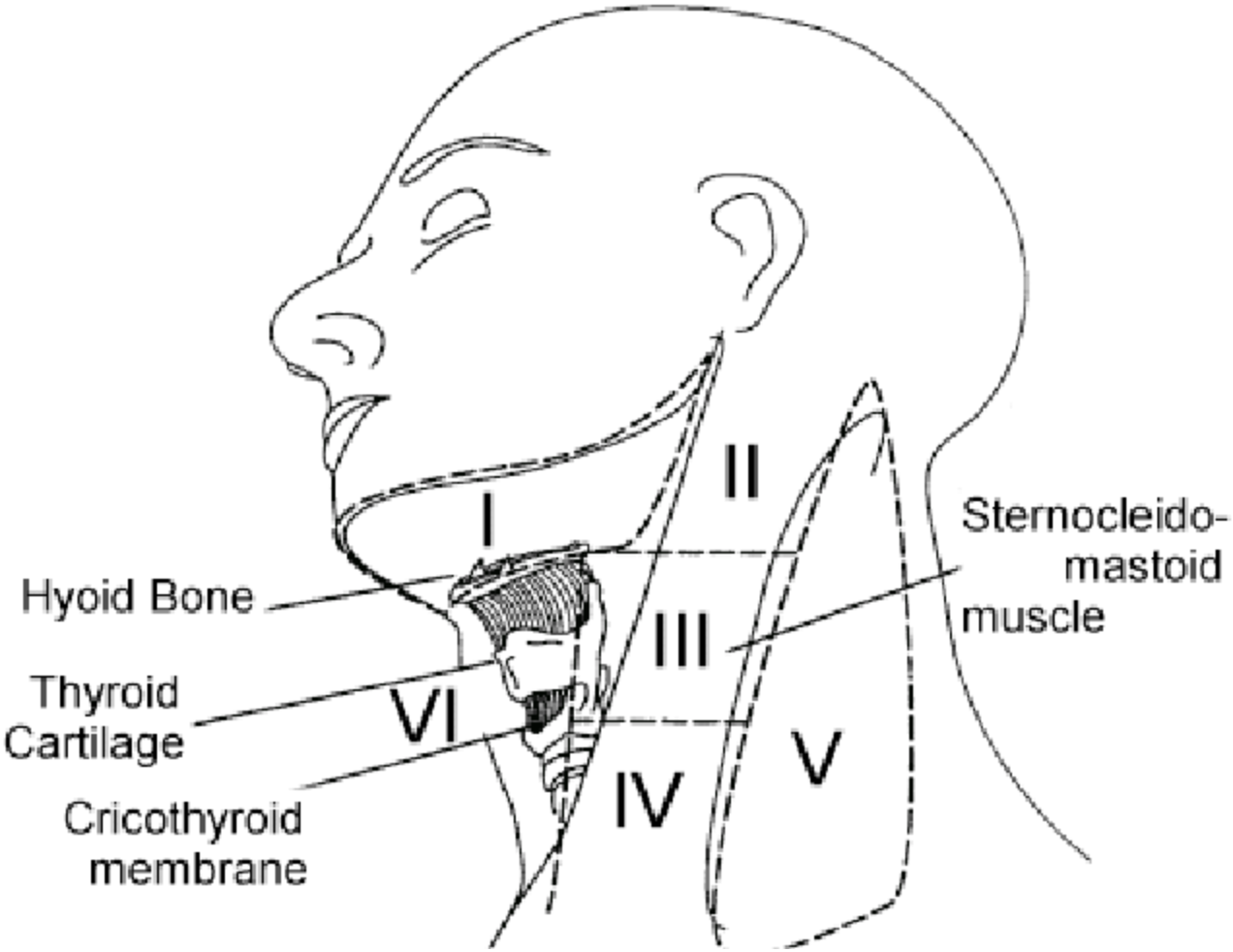
- **The growth is non-tender**
- **Size – 6 x 3 cm**
- **Shape – irregular**
- **Margin – irregular**
- **Extending anteriorly upto the anterior commissure and lies 1 cm from the upper and lower gingivobuccal sulci.**
- **Posteriorly – upto the left lower 3rd molar, lying 5 mm from the retromolar trigone**

- **There is a marked induration that is felt 5 mm beyond the margin anteriorly and 1 mm beyond the margin posteriorly.**
- **Bimanual palpation – No involvement of the mandible felt.**
- **It does not bleed on touch**
- **There was no local rise in temperature or erythema or sinus/scar/fistula**

Examination of 7th,9th,10th,11th and 12th cranial nerves was normal.

Examination of neck –

- A 4 x 4 cm single hard,fixed lymph node was palpable at level Ib on left side
- No any other swelling palpable in the neck
- Trachea is central


I
II
III
IV
V
VI
Hyoid Bone
Thyroid Cartilage
Cricothyroid membrane
Sternocleido-mastoid muscle

SYSTEMIC EXAMINATION

- **CVS – Normal heart rate and rhythm, S1 and S2 sounds are heard normal**
- **RS – Normal vesicular breath sounds**
- **CNS – All the reflexes were normal ,No any neurological deficit found.**
- **Per abdominal examination – normal**

PROVISIONAL DIAGNOSIS

A 62 year old chronic tobacco chewer with ulcero-proliferative growth left buccal mucosa and cervical lymphadenopathy at level Ib

most likely to be malignancy of oral cavity
Stage =cT3N2aM0

INVESTIGATIONS

- Edge Biopsy
- CECT head and neck – for staging
- FNAC of lymph node

Management

- Wide excision with a margin of 0.5 cm
- Neck dissection (preferably MRND 3)
- Radiotherapy
- Reconstruction with PMMC flap

Comprehensive Neck dissection (I-V)	Selective Neck dissection
• Radical neck dissection	• Supra omohyoid neck dissection - (I-III)
• Modified radical neck dissection 1. Type 1 (SAN) 2. Type 2 (SAN+IJV) 3. Type 3(SAN+IJV+SCM)	• Lateral neck dissection (II-IV)
	• Posterolateral (II-V)
	• Anterior/Central neck dissection (VI)



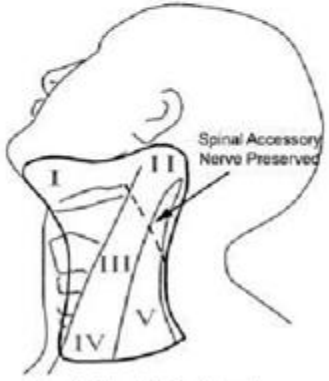
Supraomohyoid
Neck Dissection



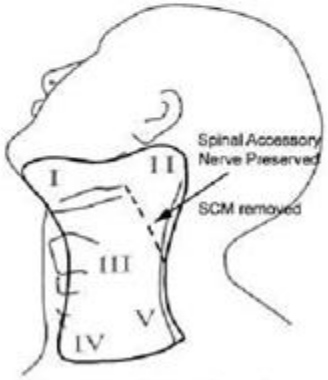
Antero-lateral
Neck Dissection



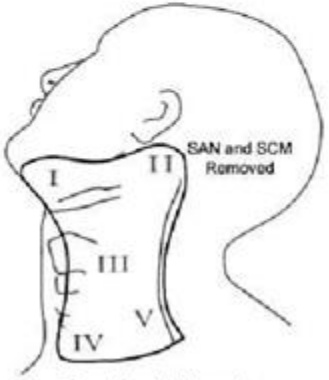
Lateral (Jugular)
Neck Dissection



Modified Radical
Neck Dissection
Type III (MRND3)



Modified Radical
Neck Dissection
TypeI (MRND1)



Radical Neck
Dissection

INDICATIONS OF NECK DISSECTION

Therapeutic	Prophylactic/ Elective neck dissection
• Clinically/Radiologically positive lymph nodes - **MRND**	• Poor grade • Thickness more than 5 mm • T2 or higher
	• Carcinoma Tongue irrespective of the stage

Contraindications of Neck dissection

- Distant metastasis
- Primary tumour beyond therapeutic range
- Fixed mass in the neck